

1.3. Schedule of Activities

The Schedule of Activities described below should be followed for all participants enrolled in Study GPHK. However, for those participants whose participation in this study is affected by exceptional circumstances (such as pandemics or natural disasters), please refer to Section 10.10 Appendix 10 for additional guidance.

1.3.1. Schedule of Activities covering visits to primary study endpoint

Visit*	1	2	3*	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	99*	ED*	801*
Week of Treatment	-2	-1	0	4	8	12	16	20	24	28	32	36	40	44	48	52	56	60	64	68	72	72		4 wks Post TxP
Allowable Deviation (days)*		±3	±3	±3	+7	±3	+7	±3	±3	±3	±3	+7	±3	±3	±3	±3	±3	±3	±3	±3	±7	±7		±3
Fasting Visit*	X	X	X	X	X	X	X	X	X			X			X			X			X	X	X	X
Telephone Visit										X	X		X	X		X	X		X	X				
Informed consent	X																							
Randomization			X																					
Register study visit in IWRS	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
Clinical Assessments																								
Medical history*	X																							
Physical examination	X																							
Height	X																							
Weight*	X		X	X	X	X	X	X	X			X			X			X			X	X	X	X
Waist circumference			X	X	X	X	X	X	X			X			X			X			X	X	X	X
Electrocardiogram*			X		X		X		X			X									X		X	X
Vital signs (3 sitting BP and HR)*	X		X	X	X	X	X	X	X			X			X			X			X		X	X
Adverse events and product complaints	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
Concomitant medications	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
Participant Education and Assessment																								
Hand out diary, instruct in use*			X			X			X			X			X			X			X			

Visit*	1	2	3*	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	99*	ED*	801*
Week of Treatment	-2	-1	0	4	8	12	16	20	24	28	32	36	40	44	48	52	56	60	64	68	72	72		4 wks Post TxP
Allowable Deviation (days)*		±3	±3	±3	+7	±3	+7	±3	±3	±3	±3	+7	±3	±3	±3	±3	±3	±3	±3	±3	±7	±7		±3
Fasting Visit*	X	X	X	X	X	X	X	X	X			X			X			X			X	X	X	X
Telephone Visit										X	X		X	X		X	X		X	X				
Review study participant diary, including study-drug compliance*				X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X		X	
Lifestyle Program instructions*			X	X	X	X			X			X			X			X			X		X	
Review diet and exercise goals*			X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X		X	
Injection training with autoinjector demonstration device*			X																					
Dispense study drug			X	X	X	X	X	X	X			X			X			X			X*			
Observe participant administer study drug*			X																					
Participant returns study drugs and injection supplies				X	X	X	X	X	X			X			X			X			X		X	
Laboratory Tests																								
Serum pregnancy test*	X																							
Urine pregnancy test*			X			X			X			X			X			X			X	X	X	
Follicle-stimulating hormone test*	X																							

Visit*	1	2	3*	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	99*	ED*	801*
Week of Treatment	-2	-1	0	4	8	12	16	20	24	28	32	36	40	44	48	52	56	60	64	68	72	72		4 wks Post TxP
Allowable Deviation (days)*		±3	±3	±3	+7	±3	+7	±3	±3	±3	±3	+7	±3	±3	±3	±3	±3	±3	±3	±3	±7	±7		±3
Fasting Visit*	X	X	X	X	X	X	X	X	X			X			X			X			X	X	X	X
Telephone Visit										X	X		X	X		X	X		X	X				
2-hour oral glucose tolerance test (includes glucose, insulin, c- peptide at each timepoint)*		X																			X		X	
Chemistry panel (include Cr for eGFR calculation and glucose)*	X		X			X			X			X			X			X			X		X	X
Lipid panel	X		X						X												X		X	X
Fasting insulin						X			X			X			X			X			X		X	X
C-peptide						X			X			X			X			X			X		X	X
Free fatty acids	X		X						X												X		X	X
Urinary albumin/creatinine	X*								X												X		X	X
Cystatin-c	X*								X												X		X	X
Calcitonin	X*					X			X												X		X	X
Hematology	X*					X			X												X		X	X
HbA1c	X		X			X			X			X			X			X			X		X	X
Pancreatic amylase, lipase	X		X			X			X												X		X	X
Thyroid-stimulating hormone	X*																							
Immunogenicity (includes PK sample)*			X	X		X			X						X						X		X	X
TZP PK*					X		X					X												
Pharmacogenetic stored sample			X																					

Visit*	1	2	3*	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	99*	ED*	801*
Week of Treatment	-2	-1	0	4	8	12	16	20	24	28	32	36	40	44	48	52	56	60	64	68	72	72		4 wks Post TxP
Allowable Deviation (days)*		±3	±3	±3	+7	±3	+7	±3	±3	±3	±3	+7	±3	±3	±3	±3	±3	±3	±3	±3	±7	±7		±3
Fasting Visit*	X	X	X	X	X	X	X	X	X			X			X			X			X	X	X	X
Telephone Visit										X	X		X	X		X	X		X	X				
Nonpharmacogenetic stored sample			X			X			X						X						X		X	X
Mental Health Questionnaires																								
PHQ-9*	X		X			X			X			X			X			X			X	X	X	X
C-SSRS (Baseline/Screening) Version)*	X																							
C-SSRS (Since Last Visit Version)*		X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
Self-Harm Supplement Form*	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
Self-Harm Follow-up Form*	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
Patient Reported Outcomes																								
PGIS			X																		X		X	
SF-36, version 2, acute form			X																		X		X	
IWQOL-Lite CT			X																		X		X	
EQ-5D-5L			X																		X		X	

1.3.2. Schedule of Activities for additional 2-year treatment period for participants with prediabetes at randomization

Visit*	101	102	103	104	105	106	107	108	109	110	111	112	113	114	115	116	199*	ED*	802*
Week of Treatment	78	85	91	98	104	111	117	124	130	137	143	150	156	163	169	176	176		17 wks Post TxP
Allowable Deviation (days)*	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7		±3
Fasting Visit*		X		X		X		X		X		X		X		X	X	X	X
Telephone Visit	X		X		X		X		X		X		X		X				
Register study visit in IWRS	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
Clinical Assessments																			
Weight*		X		X		X		X		X		X		X		X	X	X	X
Waist circumference		X		X		X		X		X		X		X		X	X	X	X
Electrocardiogram*								X								X		X	X
Vital signs (3 sitting BP and HR)*		X		X		X		X		X		X		X		X		X	X
Adverse events/Product Complaints	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
Concomitant medications	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
Participant Education and Assessment																			
Hand out diary, instruct in use*		X		X		X		X		X		X		X					
Review study participant diary, including study drug compliance*	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X		X	
Lifestyle Program instructions*		X		X		X		X		X		X		X		X		X	
Review diet and exercise goals*	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X		X	
Dispense study drug		X		X		X		X		X		X		X					

Visit*	101	102	103	104	105	106	107	108	109	110	111	112	113	114	115	116	199*	ED*	802*
Week of Treatment	78	85	91	98	104	111	117	124	130	137	143	150	156	163	169	176	176		17 wks Post TxP
Allowable Deviation (days)*	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7		±3
Fasting Visit*		X		X		X		X		X		X		X		X	X	X	X
Telephone Visit	X		X		X		X		X		X		X		X				
Participant returns study drugs and injection supplies		X		X		X		X		X		X		X		X		X	
Laboratory Tests																			
Urine pregnancy test*		X		X		X		X		X		X		X		X		X	
2 hour oral glucose tolerance test (includes glucose, insulin, c-peptide)*								X								X	X	X	X
Chemistry panel (includes Cr for eGFR calculation and glucose)*		X		X		X		X		X		X		X		X		X	X
Lipid panel								X								X		X	X
Free fatty acids								X								X		X	X
Urinary albumin/creatinine ratio								X								X		X	X
Cystatin-c								X								X		X	X
Calcitonin								X								X		X	X
Hematology								X								X		X	X
HbA1c		X		X		X		X		X		X		X		X		X	X
Pancreatic amylase, lipase								X								X		X	X
Immunogenicity (includes PK sample)*				X				X				X				X		X	X
Nonpharmacogenetic stored sample								X								X		X	X
Mental Health Questionnaires																			
PHQ-9*		X		X		X		X		X		X		X		X	X	X	X

Visit*	101	102	103	104	105	106	107	108	109	110	111	112	113	114	115	116	199*	ED*	802*
Week of Treatment	78	85	91	98	104	111	117	124	130	137	143	150	156	163	169	176	176		17 wks Post TxP
Allowable Deviation (days)*	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7	±7		±3
Fasting Visit*		X		X		X		X		X		X		X		X	X	X	X
Telephone Visit	X		X		X		X		X		X		X		X				
C-SSRS (Since Last Visit Version)*	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
Self-Harm Supplement Form*	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
Self-Harm Follow-up Form*	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
PROs																			
PGIS								X								X		X	
SF-36, version 2 acute								X								X		X	
IWQOL-Lite -CT								X								X		X	
EQ-5D-5L								X								X		X	

*Please see table below for corresponding additional information

Abbreviations: BP = blood pressure; Cr= creatinine; C-SSRS = Columbia-Suicide Severity Rating Scale; ED= early discontinuation of treatment; eGFR = estimated glomerular filtration rate; HbA1c = hemoglobin A1c; HR = heart rate; IWQOL-Lite-CT = Impact of Weight on Quality of Life-Lite Clinical trials; IWRS = interactive web-response system; PGIS = Patient Global Impression of status for physical activity; PHQ-9 = Patient Health Questionnaire-9; PK = pharmacokinetics; SF-36 = Short-Form-36 Health Survey version 2, acute form; TxP = treatment period; TZP = tirzepatide

Additional Information Regarding Activities Described in Schedule of Activities

Activity	Notes
Visit Fasting Visit	On all office visits, study participants should be reminded to report to the site before taking study drug (s) in a fasting condition, after a period of approximately 8 hours without eating, drinking (except water), or any significant physical activity. Since some screening procedures need to be completed in the fasting state, Visit 1 may be conducted over more than 1 day to ensure necessary conditions are met.
Visit 3	Baseline assessments must be completed before processing in the IWRS.
Visit 99 Visit 199	Participants wanting to discontinue the study before Week 72 will be asked to return for Visit 99 at 72 weeks $\pm$ 7 days after randomization primarily for body weight measurement and assessment of adverse events. If the participant is unwilling to attend Visit 99, it should be documented in the participant medical record that the participant has refused to attend. Participants wanting to discontinue the study after Week 72 but before Week 176 will be asked to return for Visit 199 primarily for the body weight measurement and assessment of adverse events. Refusal to attend should be documented.

ED	Participants who are unable or unwilling to continue the study treatment for any reason will perform an ED visit. If the participant is discontinuing during an unscheduled visit or a scheduled visit, that visit should be performed as the ED visit.
Safety Follow-up Visits Visit 801 Visit 802	For participants who discontinue or complete the study within the first 72 weeks, Visit 801 (safety follow-up visit) should be performed 4 weeks after the last visit during the 72-week treatment period. Participants with prediabetes continuing to Week 78 should not perform Visit 801. For participants with prediabetes who complete the 176 weeks or discontinue after Week 72, Visit 802 should be performed 17 weeks after the last visit during the additional 2-year treatment period for participants with prediabetes at randomization.
Allowable Deviation (days)	The visit date is determined in relation to the date of the randomization visit ($\pm$ the allowed visit window).
Medical History	Medical history includes assessment of preexisting conditions (for example, history of gallbladder disease, cardiovascular disease, and medullary thyroid carcinoma) and substance usage (such as alcohol and tobacco).
Weight	Weight measurements should be obtained per the instructions in Section 10.7. Body weight must be measured in fasting state. If the participant is not fasting, the participant should be called in for a new visit within the visit window to have the fasting body weight measured.
Electrocardiogram	ECG measurements should be obtained per the instructions in Section 10.7. ECGs will be performed in triplicate at Weeks 0 and 36. All other ECGs will be single measurements. ECGs should be collected at least 30 minutes prior to collection of blood samples for laboratory testing, including PK samples.
Vital signs (3 sitting BP and HR)	Vital-sign measurements should be taken before obtaining an ECG tracing and before collection of blood samples for laboratory testing, per the instruction in Section 10.7.
Hand out diary, instruct in use	All training should be repeated as needed to ensure participant compliance.
Review study participant diary and study-drug compliance	Data from diary, as well as study drug compliance, should be reviewed.

Lifestyle Program instructions	Counselling on diet and exercise, to be performed by a dietician or equivalent qualified delegate, to include calculation of individualized energy requirement and methods to change dietary composition and amount of physical activity. The Lifestyle Program Instruction may be delivered on a separate day from the rest of that visit's study procedures but must occur within the visit window. Beginning at Week 8, the Lifestyle Program Instruction may be delivered by phone.
Review diet and exercise goals	All training should be repeated as needed to ensure participant compliance. Study personnel to provide reinforcement and encouragement for lifestyle modifications.
Injection training with Autoinjector demonstration device	All training should be repeated as needed to ensure participant compliance
Dispense study drug Visit 72	This applies only to those participants going into the additional 2-year treatment period for participants with prediabetes at randomization.
Observe participant administer study drug	Participants should administer their first dose of study drug at the end of the Visit 3, after other study procedures are completed
Serum pregnancy test	A serum pregnancy test will be performed at Visit 1 for women of childbearing potential only.
Urine pregnancy test	A urine pregnancy test must be performed at Visit 3 with the result available prior to randomization and first injection of study drug(s) for women of childbearing potential only. Additional pregnancy tests (beyond those required per the Schedule of Activities) should be performed at any time during the trial if a menstrual period is missed, there is clinical suspicion of pregnancy, or as required by local law or regulation.
Follicle-stimulating hormone test	Follicle-stimulating hormone test performed at Visit 1 for postmenopausal women at least 40 years of age with an intact uterus, not on hormone therapy, and who have had spontaneous amenorrhea for more than 1 year without an alternative medical cause.
2-hour oral glucose tolerance test (includes glucose, insulin, c-peptide)	2-hour OGTT testing should be omitted at visits following a protocol-defined diabetes diagnosis.

Urinary albumin/creatinine ratio Cystatin-c Calcitonin Hematology Thyroid-stimulating hormone (TSH)	Screening visit assessment will be used to confirm eligibility. If calcitonin results are not available, a final review of eligibility will occur at Visit 3 before randomization can proceed.
Chemistry panel (include Cr for eGFR calculation)	The CKD-EPI equation will be used by the central lab to estimate and report eGFR.
Immunogenicity (includes PK sample)	PK samples for immunogenicity must be taken prior to drug administration at these visits. In the event of systemic drug hypersensitivity reactions (immediate or nonimmediate), additional blood samples will be collected including ADA, PK, and exploratory biomarker samples.
TZP PK	PK samples will be collected at these visits at time windows of 1 to 24 hours, 24 to 96 hours, or 120 to 168 hours post dose, as assigned by IWRS at randomization. Dependent on the time windows to which a participant is assigned, they may be required to come to site for PK-specific visits.
PHQ-9 C-SSRS (Baseline/Screening Version) C-SSRS (Since last Visit Version) Self-Harm Supplement Form	The C-SSRS, Self-Harm Supplement Form and PHQ-9 should be administered <i>after</i> assessment of adverse events. For this study, the C-SSRS is adapted for the assessment of the ideation and behavior categories only. The Intensity of Ideation and Lethality of Behavior sections are removed.
Self-Harm Follow-up Form	Self-Harm Follow-up Form is only required if triggered by the Self-Harm Supplement Form, per instructions in the form.

Abbreviations: ADA = antidrug antibody; BP = blood pressure; CKD-EPI = Chronic Kidney Disease-Epidemiology; Cr = creatinine; C-SSRS = Columbia-Suicide Severity Rating Scale; ECG = electrocardiogram; ED = early discontinuation of treatment; eGFR = estimated glomerular filtration rate; HR = heart rate; IWRS = interactive web-response system; OGTT = oral glucose tolerance test; PHQ-9 = Patient Health Questionnaire-9; PK = pharmacokinetic; TSH = thyroid-stimulating hormone; TZP = tirzepatide.